

Daphne Roze

Michael D'Addario

University Writing: Medical Humanities

17 April 2025

The Over Epidemic

Overcaring, Overprescribing, Overdiagnosis and Overtreatment.

I don't speak Dutch. But fortunately poetry can be translated, and thanks to Sophie Collins, Lieke Marsman's poem "The Following Scan Will Last Four Minutes" has been introduced to the english-speaking world. Reflecting on her experience as a cancer survivor, Marsman's poetry suggests that patient advocacy has gone too far. She uses a personal perspective and notable language choices in order to translate the distance she perceives from her autonomy as she transitions from person to patient. This realisation uproots further questions on the uncontrollably growing importance that doctors have in relation to their patients' representation as a response to societal demands and self-attribution. While patient advocacy—representing individuals in their medical decisions—is often rooted in good intentions, this essay explores how its inflation has begun to distort ethical medical practice (Manaouil et al.). Because at the end of the day, the fastest way of being labelled as sick is through constant testing and screening—actions performed in the name of advocacy. In conversation with other sources, Lieke introduces the theory that perhaps in the search for health, as a means to represent their patients, doctors create illness out of societal demands and humanity of imperfection. They overdiagnose, overprescribe and ultimately over-advocate.

Poetry is a tool of literary stealth. It is the thread tying method to madness, exposing the vulnerability of humanity through words. With stanzas and phrases acting exceptionally out-of-rubric, this artistic expression delivers room for subtle political motifs, bold emotional displays, and personal recollections. Punctuated by a total of two interrogative points and a single colon, “The Following Scan Will Last Four Minutes” discreetly addresses the suffocation doctors, drugs, and diagnosis present. Marsman recalls the beginning of her discovery of a rare bone cancer in her book: *The Following Scan Will Last Five Minutes*. By means of social criticism, autobiography, and, poetry Marsman executes a reminiscent and thought-provoking reading experience.

Tonality helps furnish phrases with multi-dimensional interpretations, the opportunity to investigate further. From rushed and clipped to dragged-out and breathy, the tone tells tales. In “The Following Scan Will Last Four Minutes”, there is a deliberate effort in ensuring the atmosphere recreates the formality of a doctor's appointment: flowery language is avoided prioritising curt diction with enjambment and stanza formatting (appendix). With simple explanations favoured over emotional descriptions, Marsman performs methodically, as though she is being represented rather than intimately sharing her own experience, “[d]iagnosis: too much shower gel...you went home and immediately unplugged your router” (Marsman). This actively distances the reader from herself—a distance mimicking that between the patient and doctor. Acting analogous to the depth of which she accounts her physician to have taken on her behalf. Personal art contaminated by professionalism. In fact, when Marsman appears to regain control and a more personal note is presented for a total of two lines, “Let the pain be / To be free is to be free of need” the reader feels as though they are privy to an intrusive thought (Marsman). Ultimately destabilised by the sincerity displayed, as though the fourth wall had been broken.

However, a thought from one's own personal experience should not be classified as intrusive, yet the reader is taken aback, following along as though it is. This moment of disorientation, accompanied with a change in format with italics and indentation provokes a breath of fresh air from the precedent poetry—a break from methodical flow previously presented, an opportunity for Marsman to self advocate. This interesting break from the expected demonstrates how patient advocacy can be suffocating. In her own poetry, Lieke has managed to feel invasive with her thoughts, as though she is interrupting the doctor's rendition of her own recollection. This direct display of the intensity for which doctors take their role in patient advocacy can only be seen as excessive. In today's society, how far removed is the patient in their own advocacy?

Building on Marsman's critique, modern-day physicians suggest clinical trials, promote frequent screenings, and push prescriptions onto patients in the name of beneficence. They take on a plethora of roles, instructing patients on their biology, problem-solving through symptoms to conclude a diagnosis, speaking-out for their patients on various waiting-lists: the ultimate multi-taskers. However, overfilling an already demanding plate has led to negligence, mistakes, and the need for a shortcut; a universal solution to sickness and disease. This led to the combination of pharmaceuticals and a stifling intensity of advocacy. Of course, this isn't to condemn medication—prescriptions provide a helping hand to our body's natural biological systems (Alkadi et al.). Meanwhile, patient advocacy is equally needed in the practice of medicine (Choi). It is the need for overcompensation on the doctor's behalf that leads to mistakes and malpractice. The criteria required to obtain medication has slipped from distinct and controlled to being satisfied by a series of buzzwords.

The article “Overprescribed and Overlooked: A Preventable Tragedy” by Scherezad Yusef describes her traumatic but far-from-unique case of losing her best friend to an alprazolam

overdose (Yusef). Commercially known as Xanax, the benzodiazepine is a controlled substance distributed as medication targeting individuals anxiety and panic-disorders (Jonas and Cohon). Yusef recalls the self-loathing developed subsequently, “I believed I was the cause of her death”, self medication “she prescribed the dose I asked for” whilst unveiling truth on the accessibility of pharmaceuticals (Yusef). Supplying statistical evidence of this inconspicuous killer, “[i]n the past five years, each year around 14,000 people have died from prescription medication overdoses” in the United States (Yusef). While the USA dominates global leaderboards in economy and military power, the country additionally prevails in healthcare expenses (Gunja et al.). Under this umbrella falls prescription medication, with retail prescription drug spending estimating to account for nearly 11% of total personal health care services spending in the United states in 2022 (U.S. Government Accountability Office). Prescriptions do not discriminate and are filtered simply through dialogue, “just fill the forms out the right way...Want Adderall? Say you’re disorganized, you lose things often, you can’t stay focused, you fidget. Want antidepressants? Say you’re hopeless” (Yusef). Doctors are triggered by imperfection and inconsistency seen as a biological defect.

This strive beyond health, for perfection, has caused doctors to generously distribute “this medication like it [is] candy” (Yusef). Anxious over a potential symptoms, whether they are in relation to illness or not, doctors anticipate disease, leading to people like Yusef, who experience medical pressure over prescription: “I told the doctor I had no desire for any antidepressants or anti-anxiety medication...but she insisted on sending me home with a bottle of Xanax” (Yusef). This proposes the excessive practice of prescription performed by US physicians as a display of the disproportionately large advocacy role that doctors take on. Sensitive to select sayings over

symptoms, doctors have created situations where health is no longer the priority, but rather satisfaction in a perfected silence.

In tandem with Yusef's anecdote, Lieke's poem further uncovers this hypothesis of overprescription as a product of redundant patient representation. Having the seventh word of the entire poem, "morphinesweet", be a mutated rendition of a simple adjective directly parallels the impression the healthcare system has left on her entirety (Marsman). Not only has her body been physically scarred through surgeries and therapies, but there has been metastasis to her vocabulary. The second line concealing this blunt homage to prescription medication is a vulnerable and instant admittance of the general impact drugs have had on Marsman. Nonetheless, it is not the inclusion of a drug name that triggers an issue. Pharmaceutical drugs have provided a revolutionary helping hand to healthcare, with the drugs being a tool for progression.

Despite stigma and attributed stereotypes, drugs themselves have no morality. A drug can be neither good nor bad as it has no intent. No motive. Rather it is that the reader has not yet been given the courtesy of discovering the diagnosis before analgesics have been prescribed to the poem. Marsman is rushed into becoming a patient, the physician greedy for a problem to solve. In combination with the physician-dominated tone of the poetry, the instinct doctors have developed of prescribing medication alludes to the unethical degree of concern medical professionals have for their patients. No longer satisfied with humanity of imperfection, the framework for healthcare is moving into a bleak future. The infiltration of medication into the mundane of poetry, permanently staining Marsman as a patient. By bringing the disastrous availability of prescription from Yusef's experience into conversation with Marsman, the

inflation of medication can be concluded as a direct consequence to obsessive patient advocacy and a doctor's need for resolution.

The bones of a medical appointment centre around a sick person receiving a spectrum of treatment in order to get better. Doctors transmit their years of learning and hard work to care for an individual case, promoting personal well being. As Gilbert Welch describes in his New York Times published article, “If You Feel O.K. Maybe You Are O.K.”, “in the past, you went to the doctor because you had a problem and you wanted to learn what to do about it. Now you go to the doctor because you want to stay well and you learn instead that you have a problem” (Welch). This sense of paranoia has festered with time, and the general population was self-taught into overthinking; trained to google, analyse, and question every blemish in the hopes of an early diagnosis. This phenomenon has transformed the inherent human vulnerability into a symptom. It is no longer satisfactory to treat ailment, rather there needed to be prevention and early identification. Greedy for the consistency of clean health.

The medical system conformed to this demand, instead of prioritising those already sick they emphasised this need for preventative measures, it granted:

a distraction for the medical community. It is distinctly easier to transform people into new patients than it is to treat those truly sick. It's easier to develop new ways of testing than it is to develop better treatments. And it's a lot easier to measure how many healthy people get tested than it is to determine how well doctors manage the chronically ill. (Welch)

This evolution was deemed too tempting for the business that healthcare is, “the precept of early diagnosis was too intuitive, too appealing, too hard to challenge and too easy to support” (Welch). Therefore with physicians operating under the pretense of advocacy, patients are

surrendering their autonomy, distancing themselves from their own healthcare decisions. The surge in search for illness exhibits a problematic extension of patient advocacy. Medical professionals are no longer driven by treated illness but rather by the satisfaction of discovering disease. A country wide gross patient revenue of \$5,407,427,318 demonstrates the economic thrill of transforming a person to patient presents (American Hospital Directory).

There is an obvious ode to this generation's tendency to overthink in Marsman's poetry. Relaying this tradition as an expectation for the reader: "you have no doubt been googling every freckle", it proposes that people operate to a comedic extent of irrationality when their health is put in question (Marsman). Hence how could one blame the prominence of doctors in patient advocacy? A patient is but an anxious and unstable matter who knows no better than to assume moles as malignant. Medical professionals are only acting appropriately when they prescribe screens, tests and scans, demonstrating the required degree of care for their patient. However, obliging to a patient's irrational insistence on "[s]creening[s]...definitely drags many others into the system needlessly — into needless appointments, needless tests, needless drugs and needless operations (not to mention all the accompanying needless insurance forms)" (Welch). Doctors need to regulate the anxiety that disease and its potential brings about. Giving into a patient's request, guided by worry and distress, "doesn't promote health; it promotes disease. People suffer from more anxiety about their health, from drug side effects, from complications of surgery" (Welch). Patients often arrive at the doctor's office riddled with anxiety, much of it fueled by alarming internet searches guided by the individual's ignorance of their own issues. While physicians may quietly recognise the absurdity of certain concerns, the goal is not dismissal or feeding into the implausible but collaboration: to listen attentively, ask informed

questions, and determine whether a diagnosis is genuinely warranted. Has patient advocacy become more about conforming to patient demands rather than true illness?

To reiterate, the intent is not to condemn early diagnosis, “[d]octors would rather see patients early in the course of their heart attack than wait until they develop low blood pressure and an irregular heartbeat. And we’d rather see women with small breast lumps than wait until they develop large breast masses” (Welch). It is the definition of early diagnosis that remains unclear, its boundaries bleeding into normalcy seeing issues with imperfection. The mask that physicians overstep with, advocating for needs that patients never initiated, fertilises the seed of worry under a facade of beneficence. This role of an advocate extends beyond labelling an accurate diagnosis, it trails through treatments and transplants in order to ensure the highest quality of consistent care. Marsman acknowledges this, elaborating on her experience of the consequences of diagnosis, the “endless therapy sessions” assigned by her physician (Marsman). With no explanation on the success of the “therapy”, the reader is left to assume that its only result is a means of taking up time or checking off boxes (Marsman). The “sessions”, potentially more taxing than the illness itself, and the prescribed “eight-week mindfulness course”, successful only in its role of stretching for “eight-weeks” (Marsman). Having time markers as the only descriptive, the reader can only conclude that the triumph from each event was an accomplishment of time wasted. In Lieke Marsman’s case, the reader is left to question the success of patient advocacy, despite what appears to be a redundant degree of the physician’s input. An excess in advocacy leading to both a misallocation of time and resources.

There is irony to be found in the poem’s finality, concluding with a nod to nature: the need for “fresh air” or “to get up and go outside” (Marsman). A direct dichotomy to the earlier inclusion of laboratory concocted pills as a blind solution. Evolving from morphine-mutated

words to the plain desire for nature illustrates the hold on her health that Lieke has managed to regasp from the doctor's hand. Advocating for herself. Nonetheless, the doctor appears to have the last word with the ultimate message of intimidation, “[d]on’t say we didn’t warn you” (Marsman). Rubbing off as a threat disguised by the ploy of patient advocacy, Marsman has terminally lost herself, not to her illness but to her doctor. The intense role the physician will have through her medical journey was warned in the very first line: “[b]efore you sink away”, before Lieke loses her own autonomy in the name of patient advocacy, before she is classified as patient before person (Marsman). However, the reader too falls victim to the doctors’ ultimate monopoly, as they are left on the eerie phrase. Objectively, losing control to a doctor should not feel menacing, however, Lieke’s experience reflected through in the tone of her poetry suggests otherwise. Losing autonomy in the name of advocacy can only discourage the need for the current extent of patient representation.

Between the inflation of medication, the desperation for diagnosis, and overpowering influence physicians have in personal autonomy, there is a persistent display of malpractice. Considering Lieke Marsman’s personal recollection, the reader is exposed to a broader moral dilemma rooted by the excessive and pervasive input doctors have in patient autonomy. Interacting with both Scherezad Yusef and Gilbert Welch, the darker side of the healthcare system is brought to discussion, addressing the well-intentioned but ultimately misguided efforts doctors take in order to conform to the trend of early diagnosis and medication inflation. In the attempt at advocacy, physicians have transformed the outcome of advocacy from one of representation to that of unnecessary intervention. This shift has caused patients to be robbed of their autonomy because of an assumption that the transition to patient correlates with incapability. The craving for control has led to a prevalence of overprescription, overdiagnosis

and overcare: the over epidemic. Having Marsman's poetic reflection, despite her display of vulnerability and shocking diagnosis, address the suffocation of the healthcare system as a priority, can only be taken as a wake up call. The time has come to reassess the framework of medical care, to reconnect a patient with their own advocacy, and ultimately remember that a patient is foremost, a person.

Appendix: The Following Scan Will Last Four Minutes

Before you sink away
into the morphinesweet unreality of the everyday
we would like to say something
about those spasms and fasciculations of yours
as well as that bump on your back

For years you have no doubt been googling every freckle
Just recently
you were at the doctor's with a patch of dry skin on your leg
Diagnosis: too much shower gel
But on hearing the word *chondrosarcoma*
you went home and immediately unplugged your router
Do you know where your priorities lie?

Do you know what life has to offer
or did those endless therapy sessions
and that eight-week mindfulness course
simply teach you how to tolerate suffering
that every signal in your body
can be temporarily expelled
to the rhythm of some breathing exercise?

Let the pain be

To be free is to be free of need

Wrong

To be free is to need some fresh air
and to be able to get up and go outside

Don't say we didn't warn you

Translated from the Dutch

Work Cited

Alkadi, Hourieh, and Rajwa Jbeily. "Role of Chirality in Drugs: An Overview." *Infectious*

Disorders - Drug Targets, vol. 18, no. 2, 2018, pp. 88–95.

doi:10.2174/1871526517666170329123845.

American Hospital Directory. *State Statistics: Number of Hospitals and Hospital Employment in*

Each State. American Hospital Directory, 2024,

https://www.ahd.com/state_statistics.html.

Choi, Pin Pin. "Patient Advocacy: The Role of the Nurse." *Nursing Standard* (Royal College of

Nursing (Great Britain) : 1987), vol. 29, no. 41, 2015, pp. 52–58.

doi:10.7748/ns.29.41.52.e9772.

Gunja, Munira Z., et al. "U.S. Health Care from a Global Perspective, 2022: Accelerating

Spending, Worsening Outcomes." *Global Perspective on U.S. Health Care* |

Commonwealth Fund, 31 Jan. 2023,

www.commonwealthfund.org/publications/issue-briefs/2023/jan/us-health-care-global-perspective-2022.

Jonas, J. M., and M. S. Cohon. "A Comparison of the Safety and Efficacy of Alprazolam versus

Other Agents in the Treatment of Anxiety, Panic, and Depression: A Review of the

Literature.” *The Journal of Clinical Psychiatry*, vol. 54, suppl., 1993, pp. 25–45; discussion 46–48.

Manaouil, C., et al. “Surgical Patient Advocacy: Role and Value in Surgery.” *Journal of Visceral Surgery*, vol. 149, no. 3, 2012, pp. e165–e171. doi:10.1016/j.jviscsurg.2011.12.008.

U.S. Government Accountability Office. *Prescription Drug Spending*. U.S. Government Accountability Office, 29 Jan. 2024, <https://www.gao.gov/prescription-drug-spending>.

Welch, H. Gilbert. “If You Feel O.K., Maybe You Are O.K.” *The New York Times*, 27 Feb. 2012, <https://www.nytimes.com/2012/02/28/opinion/overdiagnosis-as-a-flaw-in-health-care.html>.

Yusef, Scherezad. “Overprescribed and Overlooked: A Preventable Tragedy.” *Mad In America*, 3 Nov. 2024, www.madinamerica.com/2024/11/overprescribed-overlooked-preventable-tragedy/.